

ATTACHMENT B

**DELIVERY SYSTEM REFORM INCENTIVE PAYMENT (DSRIP) PROGRAM
COMMUNITY PARTNER (CP) BP3 ANNUAL REPORT RESPONSE FORM**

PART 1: BP3 ANNUAL REPORT EXECUTIVE SUMMARY

General Information

INDOOR AIR QUALITY ASSESSMENT

Furnace Brook Middle School

500 Furnace Street

Marshfield, Massachusetts



Background

Full CP NameBuilding:	<u>South Shore Community Partnership</u> Furnace Brook Middle School (FBMS)
CP Address:	<u>500 Victory Rd, Quincy, MA 02171</u> 500 Furnace Street, Marshfield, Massachusetts

Assessment Coordinated Through: Fred Russell, Director of Facilities, Town of Marshfield

Reason for Request: Odor complaints in classroom 16 and Conference Room.

Date of Assessment: October 15, 2019

Massachusetts Department of Public Health/Bureau of Environmental Health (MDPH/BEH) Staff Conducting Assessment: Cory Holmes, Environmental Analyst/Inspector, IAQ Program

Building Description: Room 16 is a general classroom located on the ground floor of the FBMS. The room contains office furniture, tile floor, painted gypsum wallboard walls and suspended ceiling tiles. It shares a common wall with general classrooms on each side and the main hallway. The Conference Room is a small room on the first floor that has been turned into office space.

Windows: Windows are openable in both areas.

BP3 Annual Report Executive Summary

This report covers the activity and use of DSRIP funds from January 1, 2020 through December 31, 2020 by the South Shore Community Partnership (SSCP). SSCP is comprised of a lead agency and an affiliated partner; Aspire Health Alliance (formerly South Shore Mental Health) is the lead agency and Spectrum Health Systems is the affiliated partner. SSCP used DSRIP investments to support technology, workforce development, administration and infrastructure, integration with ACO/MCOs, quality management, consumer advisory board, and overall program operational goals for Budget Period 3 (BP3).

In BP3, SSCP strengthened its relationships with the ACO/MCOs in the South Shore and Greater Boston region. SSCP participates in monthly meetings and high-risk rounds reviews with three key ACO/MCO partners: Tufts Atrius, BMC BACO, and Steward Health Choice Network. These monthly meetings focus on individual clients and working together to ensure the best care for each member.

SSCP has quarterly meetings with BMC MCO, Tufts MCO and Tufts BIDCO. These meetings are process oriented rather than individualized case conferences. These meetings are focused on problem solving and creating better workflows. During quarterly meetings, ACO/MCOs have been supportive in making sure documented processes are working; providing key contact people for information sharing and shared case management of members; problem-solving around issues with primary care physicians and/or workflows between ACO/MCO, PCP practices, and CPs; and generally working with SSCP to provide the most effective care for our enrollees. These meetings are also utilized for data sharing and review of outstanding care plans.

In order to monitor the quality metrics finalized by the CMS for the BH CP program, SSCP established a monthly Quality Improvement Workgroup. This work group continues to focus on utilizing the dashboard that was developed to oversee performance on key quality metrics and to better understand the areas of strength and needs of development.

Technical Assistance funds were used for projects with two TA vendors, Milliman and eHana. These projects, which were comprised of auto-tagging high risk member records and records requiring action, the creation of benchmarking reports, the calculation of certain quality metrics, and the development of detailed enrollee utilization reports, began in late BP2 and were completed in BP3.

SSCP's Consumer Advisory Board has continued to be very active and committed to supporting our program. After a brief pause during COVID, it has returned to its critical work. This board is made up of 4-8 members who changed the name to Community Advisory Board because the term "consumer" was not deemed appealing or appropriate. The board make up has fluctuated due to the virtual nature and not all members being able to join virtually. It is expected when meetings can be held in person that this will no longer be a problem.

In order to manage changes in the BH CP program and monitor quality measures, SSCP has found it extremely helpful to build designated teams and workgroups. In addition to the monthly Leadership and Quality Management Team and the Hub Management Teams, SSCP has assembled a Billing Team and Quality Improvement Workgroup. Through the work of these teams, SSCP has been able to centralize procedures for managing denials, conduct research for re-enrollment of members, improve documentation, and monitor achievement on key quality measures.

Key lessons learned in BP3 are the importance of fostering strong relationships with ACO/MCOs to provide integrated care for our mutual members, building a designated workgroup to monitor quality measures, and developing internal teams and procedures to manage changes in the program. As part of our culture of continuous improvement, we have maintained our constant research into and dissemination of best practices as this program continues to develop.

Background

~~Note that this building has been visited by the MDPH IAQ Program in June 2019 at the request of the BPS to investigate similar odors. The previous assessment was prompted by odor complaints, described by occupants as a "chemical, glue-type odor". School/Town maintenance staff believed that the~~

odor was related to roofing compounds/materials. The report from that visit can be found at: <https://www.mass.gov/info-details/indoor-air-quality-reports-cities-and-towns-m>.

Methods

Please refer to the IAQ Manual and appendices for methods, sampling procedures, and interpretation of results (MDPH, 2015).

Results and Discussion

The following is a summary of indoor air testing results for *Total Volatile Organic Compounds (TVOCs)*:

- ~~Conference Room: No measurable levels of TVOCs were detected in the breathing zone at the time of assessment; a slight reading of 2.4 ppm was measured in the utility hole above the ceiling (Picture 1).~~
- ~~Room 16: No measurable levels of TVOCs were detected in the breathing zone (note that the univent was operating and bringing in fresh air at the time of testing). A slight reading of 1.4 ppm was measured near the sealed skylight above the ceiling (Picture 2).~~

Ventilation/Odors

~~A heating, ventilating and air conditioning (HVAC) system has several functions. First it provides heating and, if equipped, cooling. Second, it is a source of fresh air. Finally, an HVAC system will dilute and remove normally occurring indoor environmental pollutants by not only introducing fresh air, but also filtering the airstream and ejecting stale air to the outdoors via exhaust ventilation. Even if an HVAC system is operating as designed, point sources of respiratory irritants may be present and produce symptoms in sensitive individuals. The following analysis examines and identifies components of the HVAC system and likely sources of respiratory irritants found in the indoor environment.~~

Mechanical ventilation for classroom 16, and most general classrooms, is provided by a unit ventilator (univent) located near classroom windows (Picture 3). Univents draw air from the outdoors through a fresh air intake located on the exterior wall of the building (Picture 4) and return air through an air intake located at the base of the unit. Fresh and return air are mixed, filtered, heated or cooled and provided to rooms through an air diffuser located in the top of the unit (Figure 1). It is important to note that the univent was reportedly deactivated for several days prior to the odor complaint. It was also reported that once the univent was reactivated odors began to lessen. Not only will operating the univent

~~dilute odors by bringing in fresh/outside air but will also *pressurize* the room keeping odors out (e.g., above the ceiling tiles/away from occupants).~~

~~Most classrooms with mechanical ventilation typically have exhaust or return vents installed to remove naturally occurring airborne pollutants and provide air exchange. Room 16 does not have any exhaust/return vents. The Conference Room does have an operable mechanical exhaust vent mounted in the ceiling (Picture 5). BEH/IAQ Program staff recommended installing a vent in the exhaust ductwork above the ceiling (Picture 6) to depressurize the ceiling plenum in relation to the Conference Room, which will capture odors (if present) before they reach the occupied area.~~

Conclusions and Recommendations

The following recommendations were made at the time of the visit and are reiterated below:

- ~~1. Ensure skylight is sealed as airtight as possible using caulking or other appropriate airtight sealant. Extend this covering over the metal skylight frame (shown in Picture 2).~~
- ~~2. Operate univent in Room 16 *continuously* during occupied hours and utilize openable windows to *pressurize* the room in relation to the ceiling plenum.~~
- ~~3. Consider installing a vent or approximate one by drilling holes in the exhaust duct above the ceiling in the Conference Room to *depressurize* the ceiling plenum, which will capture odors (if present) and remove from building.~~
- ~~4. Seal any/all open utility holes in walls and ceilings in both the Conference Room and Room 16.~~
- ~~5. If odors persist, work with roofing contractor, HVAC specialist and/or an Industrial Hygienist to examine more permanent solutions to remove and/or eliminate source of odors.~~
- ~~6. Refer to resource manual and other related IAQ documents located on the MDPH's website for further building wide evaluations and advice on maintaining public buildings. These documents are available at <http://mass.gov/dph/iaq>.~~

References

MDPH. 2015. Massachusetts Department of Public Health. Massachusetts Department of Public Health Indoor Air Quality Manual: Chapters I-III. Available at: http://www.mass.gov/eohhs/gov/departments/dph/programs/environmental_health/exposure-topics/iaq/iaq-manual/.

Picture 1



Open utility hole in Conference Room

Picture 2



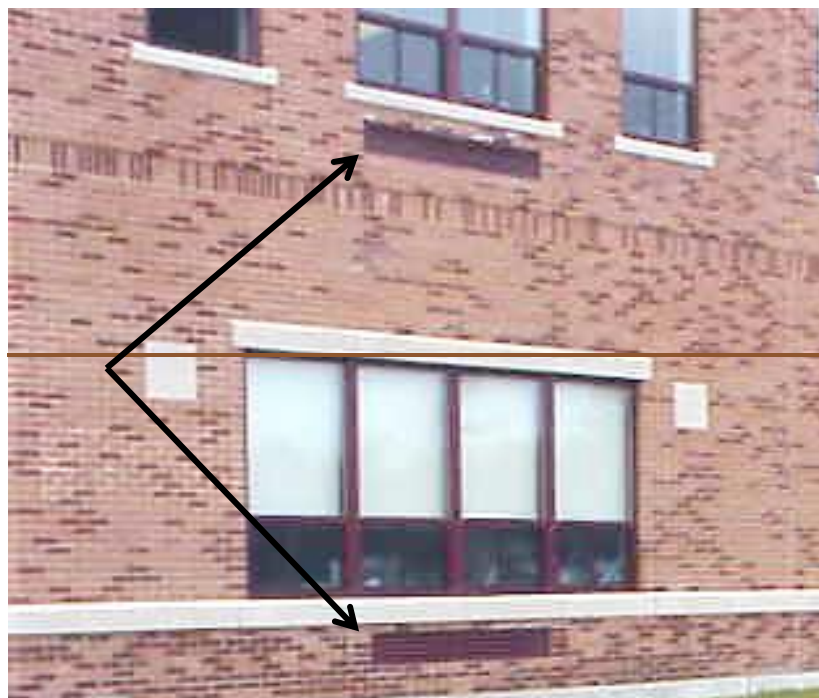
Sealed skylight above ceiling in Room 16, note metal skylight frame (arrow)

Picture 3



Classroom 16 univent

Picture 4



Univent fresh air intakes (arrows)

Picture 5



Ceiling-mounted exhaust vent in Conference Room

Picture 6



Exhaust ductwork above ceiling in Conference Room